



MVAIR

MEDVOICE AI RECEPTIONIST

INTERNAL BRIEF

Competitor Landscape & What It Means For Us

A learning brief, not an exhaustive report.

Read once, get a clear head, act.

As of June 2026. Competitor features and prices change fast — verify before quoting in a pitch.

THIS BRIEF COVERS

Part 1 — Who the competitors actually are

Part 2 — How we compete when everyone rents

Part 3 — What this means we have to do

One-line takeaway

Who the competitors actually are

There are three layers in this market. Knowing which layer someone sits in tells you whether they're a competitor, a peer, or a supplier.

LAYER 1 Infrastructure (the plumbing) — NOT our competitors

Vapi · Retell · Synthflow · Bland

These rent out the raw voice stack — speech-to-text + LLM + text-to-speech + telephony, glued together with interruption handling and low latency. We rent from Vapi. A competitor could rent from Retell and build a MedVoice clone in a weekend. So these aren't rivals — they're the supply layer we all stand on. Nobody in this market builds their own voice models; that's a multi-year infrastructure problem, and stitching it from scratch costs ~\$25K–\$60K and 3–6 months even on top of these platforms. Renting is the normal, correct model.

Note for us specifically: of these, Retell bundles HIPAA + a BAA with no enterprise contract (~\$0.07/min), while Vapi charges a flat ~\$1,000–\$2,000/month HIPAA add-on. This is the single biggest cost difference affecting our early margins — covered in Part 2.

LAYER 2 Horizontal AI receptionists — Loose competitors

My AI Front Desk · Goodcall · AINORA · Voicify

Same business model as us: rent infrastructure, wrap it in a product, sell to businesses. The difference from us is only packaging and focus — most serve many industries with a "dental template" rather than going deep. My AI Front Desk is the cheapest entry point (~\$65–\$200/mo) but trades away dental/medical depth for breadth. These compete with us on price at the shallow end, but they don't own the clinic workflow.

LAYER 3 Vertical, clinic-native products — Our real competitor

Arini is the one to study

Arini

YC W24

Company	YC W24, ~\$500K seed; founded 2024; ~15 staff; San Francisco
Focus	Dental-specific. Deployed across hundreds of practices/DSOs.
Published results	\$56K in booked appointments in one clinic's first month; 12% revenue lift at another
PMS integrations	Open Dental, Eaglesoft, Dentrrix, Denticon, CareStack, Cloud9 — writes appointments directly into the practice's system, no manual step
Capabilities	~300ms response latency · emergency triage · insurance verification · recall campaigns · SMS confirmations
Compliance	"100% HIPAA," BAAs with all subcontractors, a Trust Center — notably does <i>not</i> publicly list SOC 2 Type II
Go-to-market	30-day monitored pilot, gradual cutover. Pricing: mid-three-figures/month per location, quote-only

Others in this tier: **Viva AI, TensorLinks, Dentina, TrueLark, Peerlogic** — varying on omnichannel (email/web chat), languages, payments, and SOC 2.

Hyro is in a category of its own: ~\$95M raised, 45+ health systems, 30M+ patients, deep Epic integration, and a proprietary engine (it doesn't just rent). It's enterprise-only — not our competitor, but proof of where the ceiling is.

So how are we even "competitors" if everyone rents?

This was your question, and it's the most important idea in this whole brief:

Renting the voice layer is normal. It is not what separates winners from losers. What separates them is everything built on top of the rented layer.

Arini rents plumbing just like we do. Their advantage isn't voice quality — it's:

- **Deep PMS integration** (writes straight into Dentrix/Open Dental).
- **A complete compliance chain** (BAAs everywhere, Trust Center).
- **Funding, a team, and named references.**

That's the competition. Not "whose AI sounds more human" — they all sound fine now, all using the same neural voices. The fight is over **workflow depth, compliance, and trust**. Voice quality has been commoditized away from everyone.

The uncomfortable read: today we are a thinner version of Arini — we book to Google Calendar, not to a real PMS, and we don't yet have a BAA chain. We're at Layer 2 trying to reach Layer 3.

What this means we have to do

Given where we sit, four moves — in order.

1

Settle the voice-provider economics (decide first, it gates everything)

Vapi's ~\$1,000–\$2,000/mo HIPAA add-on makes our first 1–5 clinics lose money. It only turns healthy past ~10 clinics. Retell includes HIPAA + BAA with no fixed floor. Before building anything else, decide: absorb Vapi's HIPAA cost as a launch expense, or migrate the voice layer to Retell. **This is an economic decision, not a feature.**

2

Build the moat we don't have: one real PMS integration

Google Calendar is not a moat — Arini writes into the actual practice system. Pick one PMS we can realistically integrate (**Open Dental has the most accessible API**; Jane App's is approval-only and currently closed) and integrate it. PMS integration is simultaneously (a) the only durable moat, and (b) the #1 defense against churn — ~70% of SMB SaaS churn happens in the first 90 days, and being embedded in the clinic's core system makes us "too critical to rip out."

3

Close the compliance chain so we can legally charge

Signed BAA with the clinic plus flow-down BAAs with every subcontractor touching patient data. Add the recording proxy and secure auth. **Without this we cannot legally take money from a clinic — full stop.**

4

Don't try to out-feature Arini. Out-focus them.

We will not win on breadth or budget against a funded YC team. We win by going narrow — one specialty, one PMS, one delighted reference clinic — then expanding by referral. Match the proof structure the market already expects: a free 30-day monitored pilot, after-hours first, gradual cutover. That's exactly how Arini sells; meet the bar, don't reinvent it.

THE ONE-LINE TAKEAWAY

Everyone rents the voice. The product is what you build on top — **integration + compliance + trust**. Arini has it; we don't yet. Our entire near-term job is to get **one paying, BAA-covered clinic live and wired into its PMS** — that single proof point is the moat, the churn defense, and the thing that makes us a real competitor instead of a demo.